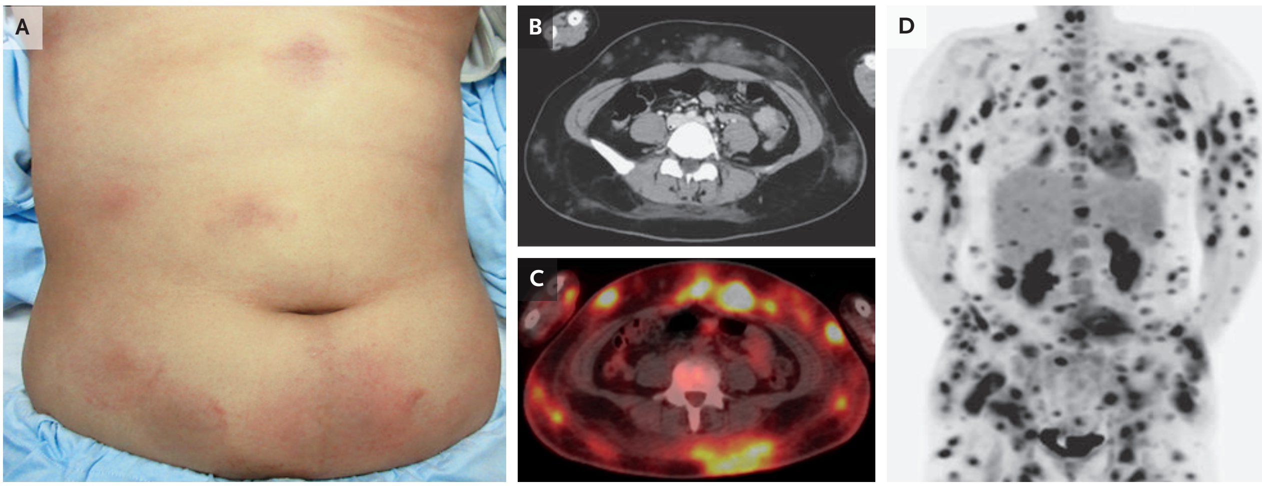
IMAGES IN CLINICAL MEDICINE

Subcutaneous Panniculitis-like T-Cell Lymphoma



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A previously healthy 30-year-old woman was admitted to the hospital with a 10-week history of painful skin nodules and a 6-week history of fevers and night sweats. A nodule biopsy performed 3 weeks before admission had shown only panniculitis. Physical examination was notable for palpable subcutaneous nodules with overlying erythema on the trunk (Panel A), arms, and legs. Computed tomography (CT) of the abdomen showed multiple areas of increased density in subcutaneous fat (Panel B). Splenomegaly was noted, but no lymphadenopathy was seen. Positron-emission tomography–CT (PET-CT) of the whole body showed uptake in the subcutaneous fat lesions (Panel C, fused PET-CT image; Panel D, maximum intensity projection image). Histopathological analysis of a repeat biopsy specimen showed infiltration of inflammatory cells into subcutaneous fat tissue. Adipocytes rimmed by atypical lymphocytes (positive for CD3, CD8, and perforin 1 and negative for CD4 and Epstein–Barr virus–encoded RNA) were also seen. Clonal T-cell receptor–β gene rearrangement was identified. A diagnosis of subcutaneous panniculitis-like T-cell lymphoma — a typically indolent primary cutaneous T-cell lymphoma characterized by adipotropism — was made. Results of laboratory testing did not fulfill the clinical criteria for a diagnosis of hemophagocytic lymphohistiocytosis, which is associated with this lymphoma in some cases. Treatment with chemotherapy was given owing to the rapid progression of the lymphoma. The patient had complete remission that was maintained as of a 2-year follow-up visit.

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